Health City HOSPITAL

NABL-MC3434

হেলথ্ সিটী হাস্পতাল

DEPARTMENT OF LABORATORY MEDICINE

DEPARTMENT OF LABORATORY SERVICES

Lab No	186257	OP/IP No	Age/Sex		
UHID	94275	OP-228710	25 Years 1 Months 20 Days	Male	
Patient Name	MR ANGSUMAN DAS		Date & Time of Collection	09/03/2022	18:31:56
Doctor	Dr N/A		Date & Time of Receiving	09/03/2022	18:31:56
Ward/Bed			Date & Time of Reporting	09/03/2022	20:05:37

INVESTIGATIONS	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
BIOCHEMISTRY			
CREATININE , Serum			
CREATININE (Method:Enzymatic Amidohydrolase)	0.9	mg/dl	0.66 - 1.25
GLUCOSE RANDOM , Random Plasma (Method:Glucose Oxidase)	102	mg/dL	70 - 139
HAEMATOLOGY			
BLOOD RE , Whole Blood			
TOTAL LEUKOCYTE COUNT (WBC) (Method:Coulter Principle)	7000	/Cumm.	4000 - 10000
DIFFERENTIAL COUNT			
NEUTROPHIL (Method:VCSn Technology)	66	%	40 - 80
LYMPHOCYTE (Method:VCSn Technology)	24	%	20 - 45
MONOCYTE (Method:VCSn Technology)	4	%	02 - 10
EOSINOPHIL (Method:VCSn Technology)	6	%	01 - 06
HAEMOGLOBIN (HB) (Method:Photometric Measurement)	15.3	gm%	13 - 16.5
ESR (Method:Westergren method)	05	mm.	0 - 20mm
PLATELET COUNT , Whole Blood			
PLATELET COUNT (Method:Coulter Principle & Microscopy)	180	Thousand/cumm	150 - 430

Health City DEPARTMENT OF LABORATORY MEDICINE

----End of report----

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INVESTIGATIONS	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
SEROLOGY			
VIRAL PROFILE , Serum			
HIV I & II (Method:Chemiluminescence)	0.09 - Negative		Reactive >= 1.0 Negative < 0.90 Borderline >= 0.9 to < 1.0
Anti - HEPATITIS C VIRUS (Method:Chemiluminescence)	0.02 - Negative		Reactive >= 1.0 Negative < 0.90 Borderline >= 0.9 to < 1.0
HEPATITIS B SURFACE ANTIGEN (Method:Chemiluminescence)	0.20 - Negative		Reactive >= 1.0 Negative < 0.90 Borderline >= 0.9 to < 1.0

ANTI HIV 1 & 2

- If a specimen is reactive (result >1.00 s/c) the probability that HIV antibodies are present is high, especially in subjects at high risk for HIV infection. In most settings, it is appropriate to investigate reactive results by additional, more specific tests. Clinical correlation is indicated with appropriate counselling, medical intervention and possibly additional testing to decide whether a diagnosis of HIV infection is accurate.
- Interpretation of results from specimens found to be reactive and negative by additional, more specific tests is unclear. Further clarification may be obtained by testing another specimen obtained three to six months later.
- A negative test result does not exclude the possibility of exposure to or infection with HIV. Levels of HIV antibodies may be undetectable in the early stages of infection.

ANTI HCV

- The results from this or any other diagnostic test should be used and interpreted only in the context of the overall clinical picture.
- When Anti-HCV is detected, patient is presumed to be infected with HCV.
- A negative test result does not exclude the possibility of exposure to or infection with HCV. HCV antibodies may beundetectable in some stages of the infection and in some clinical conditions.
- Results from immunosuppressed individuals should be interpreted with caution.

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HBsAG

- The results from this or any other diagnostic kit should be used and interpreted only in the context of the overall clinical picture.
- Heterophilic, e.g. human anti- mouse, antibodies in the serum or plasma of certain individuals are known to cause interference with immunoassays.
- Individuals recently vaccinated for hepatitis B may give a transient positive result for HBsAg because of its presence in the vaccine.

----End of report----

Disclaimer

1. The department of Laboratory Medicine of Health City Hospital confirms that all tests have been assayed with highest quality standards with clinical safety and technical integrity
2. The test result may vary from lab to lab or from time to time for the same parameters for the same patient
3. The results of a laboratory test are dependent on the quality of the sample as well as the assay technology
4. This report is not subject to use for any medico-legal purpose.
5. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
6. For any uncontrolled circumstances the result may be delayed.Kindly contact department of Laboratory Medicine for any transcription error.

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